

COVID-19 DIAGNOSIS

- Suspect COVID-19 in the patient with new onset of symptoms in the last 14 days, consistent with COVID-19: fever, cough, shortness of breath (new or worse than before), sore throat, loss of sense of smell, taste abnormalities, runny/blocked nose, fatigue, chest pain, body aches, headache, diarrhoea.
- COVID-19 is more likely if current prevalence is high and the patient has not yet received a COVID-19 vaccine.

Give urgent attention to the patient with suspected COVID-19 and any of:

- Short of breath at rest or while talking
- Respiratory rate ≥ 30
- Oxygen saturation $< 92\%$ at rest, or sats drop to $< 87\%$ on exertion (walking 15-20m)
- Pulse rate > 120
- BP $< 90/60$
- Confused, agitated or decreased consciousness
- Sudden breathlessness, more resonant/decreased breath sounds/pain on 1 side, deviated trachea, BP $< 90/60$: **tension pneumothorax** likely
- Coughing up fresh blood

Manage and refer urgently →38.

Approach to the patient with suspected COVID-19 not needing urgent attention

Check if COVID-19 test should be done.

Testing strategy changes according to burden of infection - decide who is eligible for a test according to current testing strategy at your facility (check latest circular):

Universal testing

Test patients with COVID-19 symptoms referred to hospital - usually only available between waves of COVID infection, when testing capacity is not limited.

Limited testing

Test if in clusters in group settings or at risk of severe COVID-19 - usually during a 'wave' of COVID -19 infections, when testing capacity is limited.

Is patient part of a cluster in group settings such as old age/care homes, hospitals, or prisons?

Yes

No

Is patient a health care worker or does patient have any of these risk factors for severe COVID-19?

- > 60 years old
- Diabetes
- Obesity (BMI¹ ≥ 30)
- Heart disease
- HIV (if not on ARVs)
- TB (current or previous)
- Chronic kidney disease
- Chronic lung disease (like asthma, COPD)
- Cancer

Yes

No

Test for COVID-19.

- If possible, collect 2 upper respiratory swabs, preferably nasopharyngeal swabs: one rapid antigen test swab for on-site testing and one swab to send to laboratory for PCR testing if rapid antigen test negative.
- If symptoms include cough, fever or fatigue, also test for TB: send 1 sputum sample for TB NAAT → 92.

Rapid antigen test positive

- Diagnose **acute COVID-19** → 41. No need to send PCR swab.
- Notify.

Rapid antigen test negative

- If currently high number of COVID-19 cases ('wave'): send confirmatory PCR swab.
- If currently low number of COVID-19 cases: consider patient COVID-19 negative.

Rapid antigen test unavailable/inconclusive

Send swab for PCR testing.

If PCR swab sent, advise to isolate and manage as presumptive COVID-19 until PCR swab result are back →41.

- Explain that capacity for COVID-19 testing is limited and based on his/her symptoms, it is likely that s/he has COVID-19 infection.
- If symptoms include cough, fever or fatigue, also test for TB: send 1 sputum sample for TB NAAT → 92.
- Manage empirically for likely **COVID-19** →41.

¹BMI = weight (kg) ÷ height (m) ÷ height (m).